

MULTIVITAMINS +ZINC

ENERVON[®] Z+

Film-coated Tablet

Vitamins / Mineral

FORMULATION

Each film-coated tablet contains:

Ascorbic acid (Vitamin C)	500 mg
(equivalent to 568.11 mg of Sodium Ascorbate)	
Thiamine Mononitrate (Vitamin B ₁)	50 mg
Riboflavin (Vitamin B ₂)	20 mg
Niacinamide (Vitamin B ₃)	20 mg
Calcium Pantothenate (Vitamin B ₅)	20 mg
Pyridoxine Hydrochloride (Vitamin B ₆)	5 mg
Cyanocobalamin (Vitamin B ₁₂)	4 mcg
d-Alpha-Tocopheryl Acid Succinate (Vitamin E)	11 IU
Zinc Sulfate	10 mg
(equivalent to 27.5 mg Zinc Sulfate Monohydrate)	

PRODUCT DESCRIPTION

This product is an orange, modified oval, biconvex, film-coated tablet, plain on both sides

WHAT IS IN THE MEDICINE?

This medicine contains B-complex vitamins (B₁, B₂, B₅, B₁₂, Niacinamide and Calcium Pantothenate) to help optimize the conversion of food into energy that the body can utilize for numerous physiologic processes such as respiration, digestion, blood circulation, immune system response, and as fuel for physical activities.

It also has vitamins C, E and zinc which together help the body's natural defense against damaging free radicals (antioxidant effect) and help boost immune function. Free radicals are highly reactive and unstable chemicals generated during normal body activities that require oxygen (e.g., respiration, digestion, blood circulation, immune system response, increased physical activity, etc.) and after exposure to UV light, cigarette smoke and various pollutants.

STRENGTH OF THE MEDICINE

See Formulation

WHAT IS THE MEDICINE USED FOR?

This medicine is used for the prevention and treatment of vitamin and mineral deficiencies.

HOW MUCH AND HOW OFTEN SHOULD YOU USE THIS MEDICINE?

Orally, 1 tablet once a day.

Or, as directed by a doctor.

WHEN SHOULD YOU NOT TAKE THIS MEDICINE?

If you are allergic to any ingredient of the product.

UNDESIRABLE EFFECTS

Riboflavin (Vitamin B₂)

Renal and urinary disorders: Yellow-orange discoloration to urine

Niacinamide

Nervous system disorders: headache, hunger pain, paresthesia (sensation of burning, stinging or tingling of the skin), syncope (fainting), vasovagal attack (sudden drop in blood pressure that may cause fainting)

Eye disorders: Blurred vision**Cardiac disorders:** Heartburn, tachycardia (fast heart rate)**Vascular disorders:** Flushing (especially of the face and neck)**Gastrointestinal disorders:** Abdominal distention (bloating), diarrhea, flatulence, nausea, vomiting**Skin and subcutaneous tissue disorders:** Increased sebaceous gland activity, pruritus (itching)

Pyridoxine Hydrochloride (Vitamin B₆)

Although pyridoxine is generally considered to be relatively nontoxic, adverse neurologic effects have been reported following chronic administration of large pyridoxine dosages, e.g., long term administration of more than 200 mg daily.

Immune system disorders: Allergic reaction**Psychiatric disorders:** Somnolence (sleepiness)**Nervous system disorders:** Headache, paresthesia (sensation of tingling, pricking or numbness of the skin)**Gastrointestinal disorders:** Nausea**Hepatobiliary disorders:** Increases in serum aspartate aminotransferase (AST, a liver enzyme)**Investigations:** Decreased serum folic acid concentrations

Sodium ascorbate (Vitamin C)

Blood and lymphatic system disorders: Hemolysis [after large oral doses of vitamin C in patients with glucose-6-phosphate dehydrogenase (G6PD) deficiency]

Immune system disorders: Delayed-type allergic response**Psychiatric disorders:** Insomnia, sleepiness**Nervous system disorders:** Headache**Vascular disorders:** Flushing

Gastrointestinal disorders: Abdominal cramps, diarrhea (at a dose of 1 g or more daily), esophagitis (rare), flatulent distention, gastric discomfort, heartburn, intestinal obstruction (rare), nausea, transient colic, vomiting

Renal and urinary disorders: Increased urination (mild), kidney stone formation (associated with large dosages of vitamin C reportedly have been limited to individuals with preexisting renal disease)

Injury, poisoning and procedural complications: Erosion of dental enamel

d-Alpha-Tocopheryl Acid Succinate (Vitamin E)

Vitamin E is usually well tolerated. It has been reported to rarely cause the following undesirable effects at dosages exceeding 300 IU daily which generally disappear after discontinuing the product.

Endocrine disorders: Gonadal dysfunction, thyroid problems**Psychiatric disorders:** Emotional disturbances**Nervous system disorders:** Dizziness, headache**Eye disorders:** Blurred vision

Vascular disorders: Hypertension (increased blood pressure), thrombophlebitis (vein inflammation related to a blood clot)

Gastrointestinal disorders: Abdominal pain/intestinal cramps, diarrhea, flatulence, nausea**Skin and subcutaneous tissue disorders:** Rash**Musculoskeletal and connective tissue disorders:** Myopathy (muscle disorder)**Renal and urinary disorders:** Creatinuria (increased concentration of creatine in the urine)**Reproductive system and breast disorders:** Breast soreness/gynecomastia**General disorders and administration site conditions:** Fatigue/weakness

Investigations: Decreased serum thyroxine and triiodothyronine; increased serum creatine kinase, serum cholesterol, triglycerides, urinary estrogens and androgens

Doses of vitamin E greater than 1,000 mg (equivalent to 1,000 IU of dL-alpha-tocopheryl acetate) daily for prolonged periods have occasionally been associated with increased bleeding tendency in vitamin K-deficient patients such as those taking oral anticoagulants. It has also been suggested that it may increase the risk of thrombosis in some patients, such as those taking estrogens.

Zinc

Metabolism and nutrition disorders: Loss of appetite**Psychiatric disorders:** Drowsiness**Nervous system disorders:** Headache, metallic taste

Gastrointestinal disorders: Abdominal pain/cramps, diarrhea, dyspepsia, gastritis (inflammation of the stomach), gastrointestinal discomfort/irritation, nausea, vomiting

WHAT OTHER MEDICINE OR FOOD SHOULD BE AVOIDED WHILE TAKING THIS MEDICINE?

Niacinamide

- **Carbamazepine:** May decrease carbamazepine clearance

Pyridoxine Hydrochloride (Vitamin B₆)

- Effects of levodopa are reversed by pyridoxine (even doses as low as 5 mg daily); pyridoxine should be avoided.

- Pyridoxine (200 mg daily for 1 month) resulted in a 50% decrease in serum levels of phenobarbital and phenytoin.

- Concomitant use of pyridoxine may enhance amiodarone-induced photosensitivity reactions.

- Doses of pyridoxine greater than 5 to 10 mg/day should be avoided by those taking amiodarone.

Sodium ascorbate (Vitamin C)

- **Antacids:** Increased risk of aluminum toxicity has been noted with concomitant use; therefore, it is not recommended, especially in patients with renal insufficiency.

- **Aspirin:** Concomitant intake may lead to increased urinary excretion of ascorbic acid and decreased excretion of aspirin.

- **Disulfiram:** Prolonged administration of large doses (1 g daily) of vitamin C may interfere with the alcohol-disulfiram reaction.

- **Fluphenazine:** Concomitant administration with vitamin C resulted in decreased fluphenazine plasma level.

- **Iron:** Concomitant administration of more than 200 mg of vitamin C per 30 mg elemental iron increases absorption of iron from the gastrointestinal tract.

- **Mexiletine:** Large doses (1 g daily) of vitamin C may accelerate excretion of mexiletine.

- **Paracetamol:** Vitamin C increases the apparent half-life of paracetamol.

- **Oral contraceptives (containing estrogens):** These may reduce blood levels of ascorbic acid; large doses (>1 g) of vitamin C may increase plasma estrogen levels (possibly converting low-dose oral contraceptive to high-dose oral contraceptive); possibly breakthrough bleeding associated with withdrawal of high-dose vitamin C.

- **Warfarin:** Vitamin C has reportedly decreased the anticoagulant effect of warfarin.

- **Others:** Acidification of the urine following vitamin C administration may result in altered excretion of other drugs.

d-Alpha-Tocopheryl Acid Succinate (Vitamin E)

- **Anticoagulants:** Vitamin E reportedly may have anti-vitamin K effects; concomitant intake of anticoagulants (e.g., warfarin) and vitamin E may result in hemorrhage (bleeding).

- **Antiplatelets:** High doses of vitamin E may also potentiate the effects of antiplatelets (e.g., aspirin, clopidogrel, ticlopidine) and herbs with antithrombotic activity such as garlic and ginkgo.

- **Anticonvulsants (e.g., phenobarbital, phenytoin, carbamazepine):** These may lower plasma levels of vitamin E.

- **Oral contraceptives:** These may lower plasma levels of vitamin E.

- **Cholestyramine, colestipol, isoniazid, mineral oil, neomycin, orlistat, or sucralfate:** These may decrease/impair the absorption of vitamin E. Allow as much time as possible between doses.

- **Digoxin:** Vitamin E may reduce the requirement for digoxin. Monitoring is recommended.

- **Insulin:** Vitamin E may reduce the requirement for insulin. Monitoring is recommended.

- **Dietary fiber supplementation:** This may decrease the antioxidative effect of a supplement containing vitamin E (alpha-tocopherol).

- **Others:** Vitamin E may enhance the side effects of some drugs (i.e., amiodarone, ciclosporin, and zidovudine).

Zinc

- **Antibiotics:** Zinc decreases the absorption of certain antibiotics such as quinolones (e.g., ciprofloxacin, levofloxacin, ofloxacin) and tetracyclines (e.g., doxycycline, tetracycline). This medicine contains zinc and must be given at least 2 hours after or 6 hours before antibiotics.

- **Bisphosphonates:** Concomitant intake of zinc and a bisphosphonate (e.g., alendronate, etidronate, risedronate) may decrease the absorption of both zinc and the bisphosphonate.

- **Cholesterol medicines:** Additive effects are possible with concomitant administration; use with caution.

- **Copper or iron:** Zinc inhibits the absorption of copper and iron. Administer zinc and copper or iron as far apart as possible.

- **Penicillamine:** Zinc may reduce the absorption of penicillamine.

- **Food:** Concomitant intake of zinc and caffeine or tea may reduce zinc absorption. Foods containing high levels of phosphorus, calcium (dairy), phytates (e.g., bran, brown bread), or folic acid may also reduce its absorption.

Tell your doctor if you are taking other medicines, including herbal medicines and food supplements.

WHAT SHOULD YOU DO IF YOU MISS A DOSE?

If you miss the dose, just take the next dose and the subsequent doses at the usual recommended schedule.

Do not double the dose.

HOW SHOULD YOU KEEP THIS MEDICINE?

Keep the product out of reach and sight of children.

Store at temperatures not exceeding 30°C.

SIGNS AND SYMPTOMS OF OVERDOSAGE

Pyridoxine Hydrochloride (Vitamin B₆)

Long term (i.e., 2 months or longer) administration of large (megadose) dosages (e.g., usually 2 g or more daily) of pyridoxine can cause sensory neuropathy or neuronopathy syndromes.

Other side effects reported with megadoses of pyridoxine include peripheral neuropathy, unsteady gait (manner of walking), loss of limb reflexes/numbness and tingling in feet and hands, hyperesthesia (increased sensitivity to any stimulation), muscle weakness, impaired or absent tendon reflexes; bone pain, headache, dizziness, sleepiness, nausea, upset stomach, breast tenderness, photosensitivity on sun exposure, and exacerbation of acne.

Sodium ascorbate (Vitamin C)

Prolonged intake of vitamin C (ascorbic acid) in excess of 2 g per day may lead to nausea, abdominal cramps, diarrhea, and nose bleeds. Elevated serum glucose levels, gastrointestinal obstruction, and esophagitis have also been reported in those taking high oral doses of vitamin C.

In people with kidney disease, excess vitamin C also may contribute to oxalate-containing kidney stones. In healthy people, epidemiological studies do not support an association between excess vitamin C intake and kidney stones.

High vitamin C intakes may also boost iron absorption – useful for some, but problematic for people with hemochromatosis, a metabolic disease that cause excess iron accumulation.

Zinc

Signs of acute zinc toxicity (doses > 200 mg daily) include gastrointestinal pain/cramps, nausea, vomiting, diarrhea, loss of appetite, headache, lethargy (sleepiness) and iron status, and fever.

Prolonged zinc supplementation may cause impairment of copper and iron stores, and anemia at doses > 50 mg/day; immune deficiency, reduced blood high-density lipoprotein (HDL) levels, and gastric erosion at doses > 150 mg daily.

Long-term (average 6.3 years) ingestion of supplemental zinc (80 mg/day) has also been linked to a significant increase in hospitalizations for urinary tract infections and kidney stones.

WHAT TO DO WHEN YOU HAVE TAKEN MORE THAN THE RECOMMENDED DOSAGE?

If you have taken more than the recommended dosage, consult a doctor or contact a poison control center right away.

CARE THAT SHOULD BE TAKEN WHEN TAKING THIS MEDICINE

- Do not take more than the recommended dose.

- Do not use after the expiry date on the label.

WHEN SHOULD YOU CONSULT YOUR DOCTOR?

If any undesirable effect occurs.

For suspected adverse drug reaction, seek medical attention immediately and report to the FDA at www.fda.gov.ph AND Unilab at +632-8-UNILAB-1 (+632-8-864522-1)

for Metro Manila or toll-free +1-800-10-UNILAB-1 for provinces, or e-mail

productsafety@unilab.com.ph. By reporting undesirable effects,

you can help provide more information on the safety of this medicine.

AVAILABILITY, REGISTRATION NO. AND DATE OF FIRST AUTHORIZATION

Multivitamins + Zinc (ENERVON® Z+) - Box of 100s and 8s (in Flex foil x 4s),

and 30s (in Flex foil x 6s), DR-XY48654, 15 March 2023

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Manufactured by **UNILAB, Inc.**

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Trusted Quality Healthcare

